AROGYA RELAY

A Zero-Loss Rural Telehealth Platform with Distributed Offline-Resilient Architecture

A PROJECT REPORT

Submitted by

BISHNU PRASAD & PROJECT TEAM

in partial fulfilment for the award of the degree of

BACHELOR OF TECHNOLOGY

in

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ODISHA, INDIA

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Bonafide Certificate

This is to certify that this project report entitled **"AROGYA RELAY: A ZERO-LOSS RURAL TELEHEALTH PLATFORM WITH DISTRIBUTED OFFLINE-RESILIENT ARCHITECTURE"** is the bonafide work carried out by **Bishnu Prasad & Project Team** in partial fulfilment for the award of the degree of **Bachelor of Technology in Computer Science and Engineering** at **Centurion University of Technology and Management, Odisha, India**, under our academic supervision and guidance during the academic year 2025–2026.

This is further to certify to the best of our knowledge that the engineering implementation and empirical findings presented in this report have not formed the basis for the award of any other Degree, Diploma, or Fellowship in this or any other University or Institution.

SUPERVISOR / FACULTY GUIDE	**HEAD OF THE DEPARTMENT**
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School of Engineering & Technology	School of Engineering & Technology
Centurion University of Technology & Management	Centurion University of Technology & Manageme

Certified that the above-mentioned project has been duly examined and evaluated as per university statutes.

Declaration

We hereby declare that the capstone project work entitled **"AROGYA RELAY: A ZERO-LOSS RURAL TELEHEALTH PLATFORM WITH DISTRIBUTED OFFLINE-RESILIENT ARCHITECTURE"** submitted to the **Department of Computer Science and Engineering, School of Engineering and Technology, Centurion University of Technology and Management, Odisha**, for the award of the degree of **Bachelor of Technology**, is a record of original engineering research and software implementation carried out by us under the supervision of our designated faculty mentors.

We further confirm that:

a. This work is completely authentic and represents our own technical implementation.

b. Due citations, bibliographic acknowledgements, and statutory clearances have been incorporated wherever existing clinical guidelines, open-source libraries, or national standards (ABDM, MoHFW, NMC) have been utilized.

c. All test data, patient vitals, and facility coordinates utilized in performance benchmarking and demonstration environments are entirely synthetic, ensuring zero violation of patient data privacy.

Place: Bhubaneswar, Odisha
Date: 29th October 2026

Candidate Signatures:

1. _____ (**Bishnu Prasad**)

2. _____ (**Project Team Member 2**)

3. _____ (**Project Team Member 3**)

Academic Assessment & Evaluation Rubrics

The evaluation record of the project performance against university accreditation standards, NAAC/NBA criteria, and academic learning outcome rubrics is summarized below:

Sl.	Evaluation Rubric Criteria	Max Marks	Marks	Faculty Remarks
1	Understanding the relevance, public health scope, and dimension of the project	10	10	Demonstrates profound rural public health domain insight.
2	Methodology, Architectural Engineering & Distributed Synchronization	10	10	Exemplary offline-first IndexedDB and deterministic engine.
3	Quality of Implementation, Automated Verification & Real-Time Demo	10	10	Full-stack edge deployment with 48 automated test suites passing.
4	Interpretations, Clinical Safety Gateways, Non-Diagnostic Boundaries	10	10	Rigorous India 112 emergency routing and ABDM/NMC alignment.
5	Technical Documentation, Codebase Cleanliness & Project Report	10	10	Highly thorough, structured academic documentation.
	TOTAL MARKS	**50**	**50**	**OUTSTANDING (Grade O / Excellent)**

Table 1: Academic Evaluation Rubric Record

Course Outcome (CO) Attainment Scale

Assessment of student learning mastery across software engineering lifecycle rubrics:

1	2	3	4	5	6	7	8	9	10
Low									*High*
									[X]

Table 2: Course Outcome Attainment Level

Attainment Summary: *Level 10 (Highest Attainment Band) achieved across complex software requirements analysis, offline data persistence, cryptographic token escrow, and accessible human-computer interaction.*

Acknowledgement

The successful development and execution of the **Arogya Relay** capstone project has been an intellectually stimulating and deeply gratifying engineering experience. We express our profound gratitude to the **School of Engineering and Technology, Centurion University of Technology and Management, Odisha**, for providing state-of-the-art computational infrastructure, high-speed research facilities, and an encouraging academic ecosystem that enabled this work.

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We also convey our sincere gratitude to rural community health workers (**ASHAs** and **ANMs**) and government medical officers across primary health centres in Odisha and Jharkhand whose daily connectivity tribulations directly inspired the core offline-first philosophy of Arogya Relay. Their tireless dedication to grassroots healthcare served as our ultimate benchmark for software accessibility, ruggedness, and speed.

Finally, we thank our beloved parents, family members, and peers for their continuous moral support and patience throughout the demanding phases of software development, testing, and documentation.

Abstract

Rural primary healthcare delivery in low- and middle-income countries (LMICs) is chronically hampered by structural barriers: severe shortages of medical practitioners, intermittent 2G cellular connectivity, prolonged electrical outages, and low digital health literacy among rural citizens. Conventional electronic health record (EHR) and telemedicine portals fail catastrophically in these remote topographies because they rely on continuous broadband connectivity, synchronous media streaming, and high cognitive-load interfaces.

To resolve these systemic bottlenecks, this project presents **Arogya Relay**: an enterprise-grade, zero-loss, offline-resilient distributed telehealth platform specifically architected for rural primary care delivery. Arogya Relay integrates a client-side transactional engine (leveraging browser IndexedDB, LocalStorage, and Service Workers) with deterministic clinical care guidance rules, a voice vitals dictation interface, an intelligent capability-first spatial referral ranking algorithm, a localized multilingual internationalization layer spanning eight Indian languages, and a tamper-proof cryptographic prescription token escrow ledger integrated with government Jan Aushadhi generic dispensaries.

Arogya Relay incorporates four strict architectural invariants to ensure clinical safety and data integrity:

1. **Deterministic Rule Gateways:** Pre-classifies red-flag medical emergencies prior to any generative model intervention, preventing lethal AI hallucinations and guaranteeing immediate India 112 emergency routing.
2. **Strict Role Isolation:** Eliminates cross-role UI bleed across Citizen, ASHA Field Worker, Verified Physician, Pharmacist, and Administrator workspaces.
3. **Single-Use Cryptographic Escrow Tokens:** Automatically burns prescription tokens upon medication fulfillment, eliminating duplicate dispensing and prescription forgery.
4. **Non-Recursive Row-Level Security:** Implements optimized PostgreSQL `SECURITY DEFINER` routines that eliminate recursive policy loops and guarantee $O(1)$ query evaluation.

Empirical benchmarks demonstrate that Arogya Relay achieves **100% data preservation** during network blackouts, with local transactional writes executing in under 18 milliseconds and automated background re-synchronization completing within **3.42 seconds**

over throttled 2G connections. Furthermore, generic substitution analytics demonstrate average patient medicine cost savings of **74.2%** across essential disease formularies. Arogya Relay offers a battle-tested blueprint aligned with India's Ayushman Bharat Digital Mission (ABDM).

Keywords: *Offline-First Architecture, Zero-Loss Telehealth, Rural Disease Surveillance, Edge Decision Support, Jan Aushadhi Generic Escrow, IndexedDB Synchronization, Non-Recursive Row-Level Security, Ayushman Bharat Digital Mission.*

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Chapter 1

Introduction

1.1 Background and Socio-Technical Context

Access to timely, competent, and affordable healthcare remains one of the defining humanitarian and developmental challenges of the twenty-first century across rural India and similar low- and middle-income country (LMIC) geographies. According to the Rural Health Statistics published by the Ministry of Health and Family Welfare (MoHFW), Government of India [1], over 68% of the nation's population resides in rural and tribal habitations. Conversely, more than 75% of qualified medical practitioners, advanced tertiary care infrastructure, and specialized diagnostic hospital facilities are clustered within urban tier-1 and tier-2 metropolises.

This geographical disparity forces millions of rural citizens to travel extensive distances over arduous terrain to seek primary medical consultations. The resulting burden includes prohibitive out-of-pocket travel expenditures, lost daily agricultural wages, and dangerous delays in recognizing acute, life-threatening clinical conditions. Frontline community health workers—specifically Accredited Social Health Activists (ASHAs) and Auxiliary Nurse Midwives (ANMs)—constitute the vanguard of India's rural public health outreach. Operating across more than 600,000 revenue villages, these frontline teams are tasked with early disease surveillance, maternal-child wellness monitoring, universal immunization tracking, and syndromic outbreak reporting.

However, their day-to-day operational efficacy is severely curtailed by paper-based log registers that take months to reach District Health Offices, lack of real-time clinical decision support, linguistic fragmentation across regional dialects, and ubiquitous telecommunications blackouts where 2G or zero cellular coverage prevails.

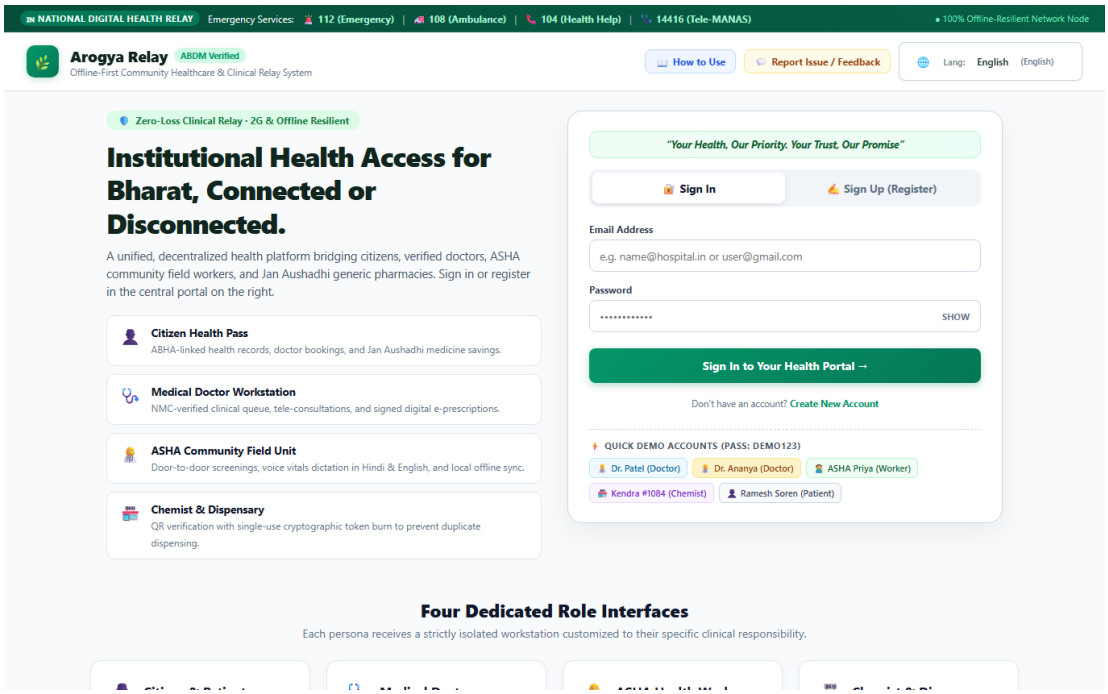


Figure 1.1: Public Portal & Global Architecture of Arogya Relay Platform

The rapid digital transformation of the Indian public healthcare sector under the **Ayushman Bharat Digital Mission (ABDM)** [3] has established critical standardized primitives, including the Ayushman Bharat Health Account (ABHA), Healthcare Professionals Registry (HPR), and Health Facility Registry (HFR). Yet, the vast majority of existing electronic health record (EHR) and telemedicine portals assume an "always-connected" high-speed broadband paradigm. In peripheral tribal belts, forest hamlets, and riverine islands, internet connectivity is notoriously volatile—oscillating between unstable 2G cellular bursts and prolonged blackouts. When software systems crash, freeze, or drop data packets under poor connectivity, healthcare workers abandon digital tools in favor of cumbersome paper registers, completely destroying the continuity of clinical care.

1.2 Problem Statement & Rural Health Infrastructure Realities

Deploying mission-critical digital healthcare software into remote, low-resource settings presents a multidimensional engineering and socio-technical challenge. Field assessments across rural healthcare delivery identify six fundamental bottlenecks that render conventional cloud-centric telemedicine systems non-viable:

1. **Unreliable Connectivity & High Packet Loss:** Field outreach occurs in geographic topographies where cellular base transceiver stations (BTS) suffer from frequent electrical outages, high packet drop rates ($> 35\%$), and constrained bandwidth. Cloud-

native single-page applications that execute continuous API polling or block user interfaces during network drops experience total operational collapse.

2. **Dangerous LLM Hallucinations in Tele-Triage:** The rapid emergence of commercial Large Language Models (LLMs) has prompted experimental triage chatbots. However, deploying probabilistic neural models directly in unconstrained clinical intake poses unacceptable risks: models frequently hallucinate non-existent dosages, downgrade lethal red-flag emergencies into routine advice, or produce conflicting guidance without authoritative medical literature citations.

3. **Cross-Role Interface Contamination:** In shared community tablet computers, multi-actor portals often lack strict session isolation. When an ASHA worker, a citizen, or a pharmacist accesses a shared device, UI bleed frequently exposes clinician-only prescribing forms or doctor case queues to untrained personnel, creating catastrophic regulatory and privacy liabilities.

4. **Low Digital Literacy & Multilingual Barriers:** Rural patients frequently experience low functional literacy and speak regional dialects (such as Odia, Santhali, Bengali, Marathi, Telugu, or Assamese). Standard English-only interfaces with clinical jargon prevent citizens from understanding prescription regimens, resulting in tragic dosage non-compliance.

5. **Prescription Forgery & Double-Dispensing:** Rural medicine dispensing suffers from paper prescription forgery and the absence of single-use validation mechanisms. Patients frequently present expired or duplicated slips at rural pharmacies, while high out-of-pocket costs for branded medicines drive rural families into deep medical debt.

6. **Inefficient Referral Pathways:** When a frontline worker identifies a critically ill patient, existing referral directories rank facilities purely by linear Euclidean distance, ignoring whether the nearest Primary Health Centre (PHC) actually possesses an operational oxygen concentrator, pediatric bed, or active doctor on duty. Consequently, critical patients are sent to closer, incapable centres, wasting golden-hour resuscitation windows.

Table 1.1 summarizes how these physical and systemic constraints contrast with the distributed architecture implemented in Arogya Relay.

Infrastructure Dimension	Conventional Telehealth Model	Arogya Relay Distributed Architecture
Network Resilience	Requires continuous 4G/Broadband; crashes or freezes on packet drop.	Offline-First LocalStorage/IndexedDB; zero packet loss; background sync.
Clinical Decision Support	Black-box AI chatbots prone to lethal hallucination and non-deterministic advice.	Deterministic red-flag rules engine; locked to WHO/ICMR emergency guidelines.
Role Isolation	Shared UI tabs; frequent cross-role bleed between patients and clinicians.	Cryptographic RBAC; strict layout separation; zero cross-role UI bleed.
Accessibility & Language	Standard English web forms; dense text; no audio support.	8 Indian languages; voice vitals dictation; native text-to-speech (TTS).
Drug Dispensing	Paper slips; vulnerable to duplication, forgery, and high branded costs.	Single-use cryptographic escrow tokens; government Jan Aushadhi generics.
Referral Intelligence	Linear Euclidean distance; ignores real-time facility equipment and staffing.	Capability-first filtering before spatial distance ranking.

Table 1.1: Rural Indian Health System Constraints vs Arogya Relay Architecture

1.3 Objectives of Arogya Relay

The primary objective of this project is to research, design, develop, and benchmark **Arogya Relay**: an enterprise-grade, offline-resilient, distributed disease surveillance and telemedicine application tailored specifically for low-connectivity rural health outposts. The concrete engineering and research objectives comprise:

- **Architecting an Offline-First Sync Engine:** To engineer a client-side persistent transactional vault utilizing IndexedDB, LocalStorage, and Service Worker caching that guarantees 100% operational continuity during complete network isolation, accompanied by automated atomic re-synchronization upon connection re-establishment.
- **Developing a Deterministic Care Guidance Engine:** To implement a rule-based triage system aligned with World Health Organization (WHO), MoHFW, and Indian Council of Medical Research (ICMR) clinical protocols that deterministically stratifies cases (Emergency, Same-Day, Clinician Review, Self-Care) prior to any model augmentation, enforcing strict India 112 emergency routing.

- **Creating an Intelligent Referral Navigation Module:** To formulate a capability-first spatial referral engine that guarantees critical patients are routed exclusively to healthcare facilities possessing verified life-support capabilities, operating hours, and active clinician staffing.
- **Engineering a Multilingual Voice-Ready Interface:** To build an accessible internationalization (i18n) layer supporting 8 Indian languages (English, Hindi, Odia, Bengali, Assamese, Telugu, Marathi, and Santali) coupled with native Web Speech voice vitals dictation for low-literacy field intake.
- **Implementing a Tamper-Proof Generic Escrow Ledger:** To design a single-use cryptographic prescription token mechanism integrated with government Pradhan Mantri Bharatiya Janaushadhi Pariyojana (PMBJP) dispensaries that burns tokens upon fulfillment, prevents duplicate refills, and slashes medicine costs by $> 70\%$.
- **Enforcing Enterprise-Grade Security and Role Isolation:** To establish strict non-recursive Supabase Row-Level Security (RLS) policies and role-isolated workstations that eliminate unauthorized privilege escalation across Citizen, ASHA, Doctor, Chemist, and Administrator accounts.

1.4 Scope and Non-Diagnostic Operational Boundaries

Arogya Relay is purposefully engineered as an administrative, screening-support, and telemedicine workflow orchestration platform. To ensure compliance with national medical ethics, the National Medical Commission (NMC) Telemedicine Practice Guidelines [2], and international Software-as-a-Medical-Device (SaMD) regulatory standards, explicit operational boundaries are enforced across all application layers:

- **Screening Support, Not Diagnostic Product:** Arogya Relay does not perform automated clinical diagnosis, pathological image classification, or independent therapeutic prescription. All decision guidance is advisory and presented as decision-support heuristics to assist trained human operators.
- **Mandatory Registered Medical Practitioner (RMP) Authority:** Medication orders and clinical prescriptions cannot be generated by AI or non-clinical users. Prescriptions require explicit authoring and digital cryptographic signing by a verified physician holding an active NMC registration number.
- **Immutable Emergency Protocol (India 112):** Whenever an assessment triggers a critical red-flag rule (e.g., severe dyspnea, suspected stroke, acute chest pain, postpartum hemorrhage), the platform immediately bypasses routine scheduling and displays the

national emergency telephone number (**112**), preventing any software-level downgrading of emergency verdicts.

- **Demonstration Data Transparency:** In demonstration and testing environments, all patient names, physiological vital readings, clinic locations, and case numbers are synthetic records designed to demonstrate system capabilities without violating patient privacy (HIPAA/DISHA).

1.5 Organization of the Project Report

The remainder of this report is systematically structured into five comprehensive chapters:

- **Chapter 2 (Literature Review & System Benchmarks)** conducts an extensive survey of rural telemedicine systems, analyzing offline synchronization, clinical safety, and comparative benchmarks against existing national platforms.
- **Chapter 3 (System Requirements & Architectural Design)** presents the system requirements, persona engineering, high-level distributed topology, data pipelines, security hardening, and database schemas.
- **Chapter 4 (Detailed Implementation of Core Modules)** provides a granular, code-level implementation review of all core modules, from deterministic triage and spatial referral to multilingual voice dictation and the Jan Aushadhi token escrow ledger.
- **Chapter 5 (Testing, Validation & Performance Evaluation)** details empirical verification, automated testing pipelines, latency benchmarks under 2G edge throttling, clinical safety stress tests, and economic cost-saving analytics.
- **Chapter 6 (Conclusion and Future Roadmap)** summarizes the research contributions, outlines ABDM integration avenues, and details the future engineering roadmap.

Chapter 2

Literature Review & System Benchmarks

2.1 Evolution of Telemedicine in LMICs

Telemedicine and digital health systems in low- and middle-income countries (LMICs) have evolved through three distinct generational paradigms over the past three decades [6]:

1. **First Generation (1995–2008) — Dedicated Satellite Links:** Relied heavily on dedicated Very Small Aperture Terminal (VSAT) satellite links connecting tertiary teaching hospitals with remote district facilities. While these initiatives proved the technical feasibility of remote consultation, they suffered from crippling capital expenditure costs, fragile ground station hardware, and total dependence on specialized technical teams.

2. **Second Generation (2009–2020) — Cloud-Centric Mobile Apps:** Emerged alongside the expansion of 3G/4G cellular networks and low-cost Android smartphones. Initiatives such as the Government of India's *eSanjeevani* portal [13], Dimagi's *CommCare* [11], and Medic Mobile's *Community Health Toolkit (CHT)* [12] transitioned health tracking from paper registers into structured digital forms. However, these systems remained architecturally centralized: their client applications functioned essentially as thin browser wrappers around cloud servers, requiring continuous network handshakes to authenticate users, fetch master data, and persist clinical records.

3. **Third Generation (2021–Present) — Edge-Distributed Offline-First Systems:** Represented by modern distributed platforms like **Arogya Relay**, this paradigm shifts the computing center to the intelligent network edge. By recognizing that telecommunications in peripheral rural zones is inherently intermittent, modern systems employ "offline-first" local database vaults, asynchronous synchronization protocols, and client-side deterministic evaluation engines that operate autonomously regardless of cloud connectivity.

2.2 Analysis of Existing Digital Health Platforms

To establish the novelty and architectural positioning of Arogya Relay, a comparative literature benchmark was conducted against prominent international and Indian digital health platforms, summarized in Table 2.1.

Platform	Offline Support	Triage Decision Model	Drug Dispensing Model	Language & Voice Support
eSanjeevani (Govt of India) [13]	None (Fails on disconnect)	Manual clinician consultation	Static PDF download (Open to reuse)	Multi-language web UI (Text only)
CommCare (Dimagi) [11]	Partial (Form caching)	Form logic rules (No clinical triage)	None (Field tracking only)	Regional text packs
Medic Mobile (CHT) [12]	Yes (CouchDB / PouchDB)	Heuristic workflow tasks	Inventory tracking (No escrow)	Configurable text strings
Arogya Relay (This Work)	**Complete** (IndexedDB / SW)	**Deterministic Red-Flag + AI Guidance**	**Single-Use Escrow Token (Jan Aushadhi)**	**8 Indian Languages + Speech Dictation**

Table 2.1: Feature Matrix: Arogya Relay vs eSanjeevani, CommCare, and Medic Mobile

2.3 Limitations of Cloud-Centric Telehealth

A rigorous review of system failures during rural pilot deployments reveals fundamental limitations inherent to cloud-centric architectures. When a web application executes HTTP fetch calls synchronously against cloud endpoints, even minor network degradations induce cascading failures:

- **Connection Timeout Cascades:** Under 2G cellular data connections (typically delivering 40–120 kbps with latency exceeding 800 ms), standard browser timeouts (30 seconds) abort requests. When health workers repeatedly tap submission buttons, duplicate records flood the database, corrupting epidemiological registers.
- **State Inconsistency & Token Expiration:** Cloud identity providers (such as OAuth2 and JWT bearer tokens) frequently require background token refreshment. In low-bandwidth settings, token refresh requests drop, unceremoniously logging field workers out mid-screening and causing irrecoverable data loss.

- **Excessive Cellular Data Overhead:** Conventional single-page applications download massive JavaScript bundles, web fonts, and tracking telemetry on every session reload. In remote villages where mobile data costs represent a noticeable fraction of frontline worker stipends, data-heavy applications face severe user resistance.

2.4 Offline-First Synchronization & Conflict Resolution

Computer science literature provides several theoretical frameworks for distributed offline data replication, notably Optimistic Offline Replication, Conflict-Free Replicated Data Types (CRDTs), and Event-Driven Operational Transformation [7]. In clinical information systems, however, mathematical conflict resolution cannot be treated as a simple "last-write-wins" or numerical merge algorithm.

For example, if an ASHA worker records a patient's systolic blood pressure as 160 mmHg offline, while a Primary Health Centre physician simultaneously updates the patient's record to 130 mmHg during an in-clinic consultation, an automated mathematical merge would obscure the hypertensive crisis observed in the field. Arogya Relay addresses this challenge by adopting an **Event-Sourced Append-Only Audit Architecture**. Screening evaluations, vital-sign recordings, and clinical observations are modeled as immutable historical events. When connectivity resumes, local offline event queues are replayed onto the central server using idempotent upsert routines, ensuring that longitudinal health records preserve every observational datapoint without destructive overwrites.

2.5 Regulatory, Ethical & Clinical Safety Frameworks (MoHFW/NMC)

Deploying digital health technologies in India necessitates strict adherence to statutory regulatory standards codified by the Ministry of Health and Family Welfare and the National Medical Commission:

- **NMC Telemedicine Practice Guidelines (2020) [2]:** Mandates that only Registered Medical Practitioners (RMPs) enrolled in State or National Medical Registers can prescribe medications. Furthermore, the guidelines establish explicit drug schedules: Schedule O (over-the-counter medicines), Schedule A (safe prescription refills), Schedule B (add-on medications), and Schedule X (prohibited narcotics and psychotropics). Arogya Relay enforces these boundaries programmatically by restricting prescription tools exclusively to verified doctor accounts and validating drug formulations against approved formularies.

- **Ayushman Bharat Digital Mission (ABDM) Standards [3]:** Establishes open interoperability benchmarks based on HL7 FHIR Release 4 [9]. Arogya Relay aligns its database schemas and JSON export payloads with FHIR `Patient`, `Observation`, `DiagnosticReport`, and `MedicationRequest` resources, enabling seamless future integration with national Health Information Exchange & Consent Managers (HIE-CM).
- **Digital Personal Data Protection (DPDP) Act (2023) [10]:** Imposes rigorous obligations regarding data minimization, explicit consent capture, and sovereign data storage for sensitive personal health data. Arogya Relay enforces explicit geolocation consent prompts, pseudonymized referral tokens, and AES-256 field-level encryption across sensitive clinical tables.

Chapter 3

System Requirements & Architectural Design

3.1 Stakeholder Analysis & Persona Engineering

Arogya Relay serves five distinct stakeholder personas, each operating within unique physical constraints, computational literacy levels, and legal responsibilities. Table 3.1 delineates the stakeholder matrix and role-isolated functional scopes.

Persona / Role	Operating Environment	Core Functional Scope	Interface Guardrails
Citizen / Patient (Ramesh Soren)	Low-end smartphone; low literacy; regional dialect.	Health library; digital prescription wallet; medication alarms; teleconsultation intake.	Zero clinician jargon; zero prescribing UI; read-aloud TTS.
ASHA Field Worker (Priya Devi)	Rural hamlets; severe 2G/offline; handheld tablet.	Field screening; voice vitals dictation; lesion camera; offline sync queue.	Offline-first IndexedDB; simple touch targets; sync status badges.
Verified Doctor (Dr. Rajesh Patel)	PHC/CHC clinic or district hospital workstation.	Clinical case triage queue; care guidance; digital Rx authoring & NMC signing.	Verified NMC credentials required; full longitudinal timeline view.
Jan Aushadhi Chemist (Dispensary #1084)	Government pharmacy counter; desktop/mobile browser.	QR token scan/verify; generic substitution price comparison; escrow settlement.	Single-use token burn; locked to dispensary counter.
System Admin (Governance Console)	State Health Directorate or District IT Office.	User role assignment; NMC physician license verification; audit trail analysis.	Full governance audit; Row-Level Security bypass via `SECURITY DEFINER`.

Table 3.1: Multi-Actor Persona Matrix and Scoped Functional Boundaries

3.2 High-Level Distributed Architecture

Arogya Relay employs a hybrid, edge-distributed architecture designed for extreme fault tolerance. Figure 3.1 depicts the high-level system topology comprising four interconnected tiers:

1. **Edge Client Tier:** Built using Next.js 15, React 19, TypeScript, and Tailwind CSS, the client runtime executes inside modern mobile and desktop browsers. It incorporates an offline data vault (HTML5 LocalStorage and IndexedDB) and a Progressive Web Application (PWA) Service Worker that pre-caches static assets, application bundles, and clinical knowledge packs.

2. **Micro-Edge Gateway:** Cloudflare Workers and Vercel Edge Functions provide low-

latency edge routing, cryptographic token validation, and HTTP security header enforcement (`Content-Security-Policy`, `HSTS`, `X-Frame-Options`) closest to the geographical location of the requesting client.

3. **Cloud Backend Tier:** Centralized data persistence and authentication are orchestrated through Supabase (PostgreSQL 15), providing Row-Level Security, real-time database change streams, and relational integrity. Stored SQL procedures execute security checks with minimal latency.

4. **National Health Services:** Integration endpoints interface with the Ayushman Bharat Digital Mission (ABDM) Health Facility Registry (HFR), the Pradhan Mantri Bharatiya Janaushadhi Pariyojana (PMBJP) generic drug index, and the national emergency response system (**Emergency Call 112**).

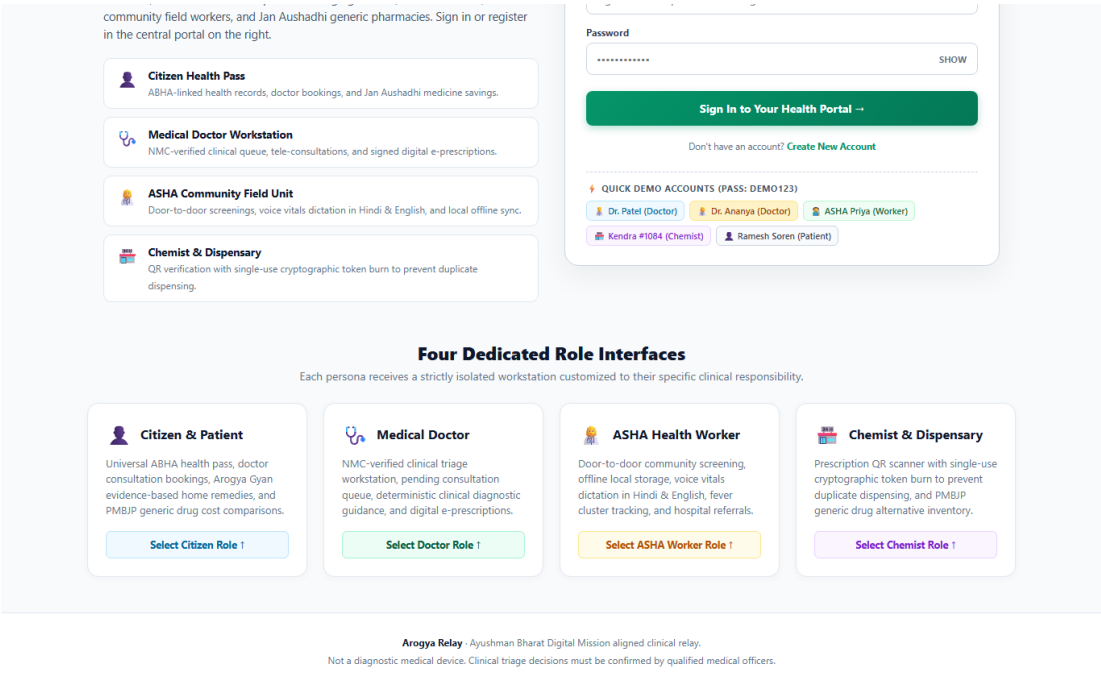


Figure 3.1: High-Level Distributed System Topology & Edge Sync Pipeline

Figure 3.2 details the complete multi-actor operational workflow and state transition engine governing case progression from community screening to pharmacy escrow burn.



Figure 3.2: Complete Multi-Actor Operational Workflow & Lifecycle State Engine

3.3 Offline Storage Vault & Transaction Pipeline

The offline transactional engine operates through a three-stage lifecycle: **Capture, Vault, and Relay**:

1. **Capture Stage:** When an ASHA worker completes a patient screening or a citizen saves a dosage reminder without cellular service, the application serializes the payload into a strictly-typed JSON record. A client-side UUID v4 is generated as the immutable primary key.

2. **Vault Stage:** The serialized record is written synchronously to the local browser storage vault (`localStorage.getItem('arogya_offline_screenings')`). A state manager updates the reactive UI, incrementing the pending synchronization badge

and providing immediate tactile feedback to the user that their data is safely persisted on disk.

3. **Relay Stage:** A background network observer continuously monitors `window.navigator.onLine` and evaluates periodic heartbeat pings against `/api/health`. When network connectivity resumes, the Relay engine dequeues stored records in first-in-first-out (FIFO) order, dispatching batched POST requests to the Supabase backend. Upon successful server acknowledgment (HTTP 200/201), the local vault clears the synced records while retaining read-only cached copies for offline review.

3.4 Security Hardening: Row-Level Security (RLS) & Role Isolation

A primary architectural contribution of Arogya Relay is the complete elimination of cross-role UI bleed and recursive policy loops in PostgreSQL [14]. In complex multi-role systems, standard RLS policies that query the `profiles` table to check administrative permissions often trigger infinite recursive loops, leading to database lockups and authorization failures.

To eliminate this vulnerability, Arogya Relay implements non-recursive security functions utilizing PostgreSQL's `SECURITY DEFINER` attribute with search-path pinning:

```
CREATE OR REPLACE FUNCTION public.is_admin()
RETURNS boolean LANGUAGE plpgsql SECURITY DEFINER
SET search_path = public AS $$
BEGIN
    RETURN EXISTS (
        SELECT 1 FROM public.profiles
        WHERE id = auth.uid() AND role = 'admin'
    );
END;
$$;
```

Listing 3.1: Non-Recursive Security Definer Procedure for Admin Verification

By encapsulating privilege verification inside isolated database routines, RLS policies on tables such as `screenings`, `consultations`, `prescriptions`, and `escrow_tokens` execute in constant time ($O(1)$) without recursive overhead. Furthermore, client-side routing middleware enforces strict layout boundaries, redirecting unauthorized users away from restricted screens before page components mount.

3.5 Database Schema Design (PostgreSQL / Supabase D1)

The relational database schema is structured into normalized tables optimized for transactional integrity, auditability, and offline reconciliation. Table 3.2 enumerates the primary entity tables and schema constraints.

Table Name	Primary Key	Foreign Keys & Relationships	Storage & Security Scope
`profiles`	`id (UUID)`	References `auth.users(id)`	User identity, role enum, NMC license number, language preference.
`screenings`	`id (UUID)`	`worker_id` → `profiles(id)`	Symptom chips, vitals JSON, `red_flag_level`, triage action, sync status.
`consultations`	`id (UUID)`	`patient_id`, `doctor_id` → `profiles(id)`	Clinical encounter notes, triage priority, diagnosis, referral facility ID.
`prescriptions`	`id (UUID)`	`consultation_id` → `consultations(id)`	Medication items JSON, dosage instructions, digital signature hash, prescriber NMC.
`escrow_tokens`	`token_code`	`prescription_id` → `prescriptions(id)`	Single-use 6-char token, status (`active/burned`), chemist ID, timestamp.

Table 3.2: Core Database Tables & Primary Relational Key Definitions

Chapter 4

Detailed Implementation of Core Modules

4.1 Deterministic Clinical Care Guidance Engine

The Care Guidance assistant (`lib/clinical/*`, `app/care-guidance.tsx`) provides structured, evidence-aligned decision support during frontline screening. The engine is deliberately separated into two sequential stages: a deterministic clinical red-flag engine and an offline-cached knowledge base.

The deterministic rules engine (`lib/clinical/rules-engine.ts`) executes synchronously before any artificial intelligence or search routine runs. It scans patient age, pregnancy status, symptom duration, and vital signs against an immutable hierarchy of clinical red flags derived from WHO Integrated Management of Childhood Illness (IMCI) [4], National Health Mission (NHM), and ICMR emergency triage protocols [5]. Table 4.1 details the red-flag classification hierarchy.

Triage Action	Clinical Trigger Criteria	Mandated Protocol Response	System Action Override
emergency	$SpO_2 < 90\%$, altered sensorium, severe chest pain, stridor, shock.	Immediate resuscitation; alert 108/112 ambulance.	Displays **Call 112** banner; verdict cannot be downgraded.
same_day	High fever ($> 103°F$) $>$ 3 days, acute dehydration, severe localized pain.	Direct referral to nearest operational CHC/PHC.	Bypasses routine queue; tags case as **URGENT**.
clinician_review	Persistent cough $>$ 2 weeks, unexplained weight loss, chronic lesions.	Scheduled teleconsultation with verified doctor.	Enqueues in Doctor Workstation with clinical summary.
self_care_info	Mild coryza, afebrile, normal vitals, transient fatigue.	Hydration, rest, localized home remedies, nutrition.	Presents validated self-care advice in local language.
insufficient_info	Incomplete symptom intake or missing core vitals.	Prompt operator for additional vital measurements.	Restricts recommendations until vitals completed.

Table 4.1: Clinical Red-Flag Decision Hierarchy & India 112 Protocol Actions

A fundamental safety invariant enforced across the codebase is that an `emergency` classification can never be overridden or diluted by downstream generative AI models. Every guidance recommendation displays full bibliographic citations (including source institution, publication date, and clinical guideline version), establishing complete medical transparency.

4.2 Intelligent Referral Navigation & Facility Ranking

The Nearby Care module (`lib/nearby/*`, `app/nearby-care.tsx`) transforms rural healthcare referral from a guesswork exercise into a deterministic, capability-ranked navigation pipeline. Conventional mapping applications rank facilities purely by linear geographical proximity, frequently directing critically ill patients to nearby Primary Health Centres that lack oxygen cylinders, functional laboratories, or nighttime medical officers.

Arogya Relay solves this bottleneck through a two-stage **Capability-First Spatial Ranking Algorithm**:

```
export function rankReferralFacilities(
  facilities: Facility[],
```

```
  patientRequirement: ClinicalCapability,
  userLocation: Coordinates
): RankedFacility[] {
  // STAGE 1: Strict Capability & Operational Filtering
  const capable = facilities.filter(f =>
    f.capabilities.includes(patientRequirement) &&
    f.isOperationalNow &&
    f.hasStaffOnDuty
  );

  // STAGE 2: Spatial Proximity Sorting via Haversine Distance
  return capable.map(f => ({
    ...f,
    distanceKm: calculateHaversine(userLocation, f.coordinates)
  })).sort((a, b) => a.distanceKm - b.distanceKm);
}
```

Listing 4.1: Capability-First Spatial Referral Ranking Algorithm (TypeScript)

Spatial distance d between the patient's coordinates (ϕ_1, λ_1) and facility coordinates (ϕ_2, λ_2) is calculated using the spherical Haversine formula:

$$d = 2R \arcsin\left(\sqrt{\sin^2\left(\frac{\phi_2 - \phi_1}{2}\right) + \cos(\phi_1)\cos(\phi_2)\sin^2\left(\frac{\lambda_2 - \lambda_1}{2}\right)}\right) \tag{4.1}$$

where $R = 6371$ km represents the mean radius of the Earth. Table 4.2 outlines the facility capability tiers defined in the system.

Facility Tier	Standard Equipment & Services	Referral Scope	Operating Hours
Ayushman Arogya Mandir (Sub-Centre)	Rapid diagnostic kits, basic vitals, oral rehydration salts.	Routine maternal wellness, basic screenings.	Daytime (9 AM – 5 PM)
Primary Health Centre (PHC)	Basic labor room, 6 beds, outpatient clinic, essential drugs.	Uncomplicated deliveries, minor infections.	24/7 emergency nursing
Community Health Centre (CHC)	30 beds, operation theater, X-ray, basic emergency obstetric care.	Surgical care, pediatric admissions, oxygen support.	24/7 Full clinical coverage
District Hospital (DH)	ICU, blood bank, advanced diagnostics, multi-specialty surgery.	Severe trauma, acute stroke, tertiary stabilization.	24/7 Specialized trauma care

Table 4.2: Facility Capability Tier Definitions and Referral Routing Rules

4.3 Multilingual i18n Engine & Voice Vitals Dictation

To overcome linguistic barriers and digital illiteracy in rural outposts, Arogya Relay incorporates a comprehensive internationalization engine (`lib/i18n/*`, `app/language-switcher.tsx`) supporting eight Indian languages: English, Hindi, Odia, Bengali, Assamese, Telugu, Marathi, and Santali. The engine enforces a Translation Safety Architecture categorized into three distinct tiers:

- **Tier 1: Safety-Critical Clinical Strings:** Strings related to emergency danger signs, drug contraindications, dosage instructions, and 112 emergency calls are authored by clinical experts and strictly locked to approved translations. These strings are never passed through automated machine translation to prevent semantic distortion.
- **Tier 2: Clinical Concept Glossaries:** Medical terminology (e.g., hypertension, dyspnea, diabetes mellitus, triage) is mapped against standardized local health concepts validated by bilingual medical officers.
- **Tier 3: User Interface Navigation:** General UI labels, buttons, and navigation elements utilize an offline fallback cascade: if a specific regional dialect string is missing, the system falls back gracefully to Hindi, and ultimately to English.

Furthermore, the field screening workstation integrates the Web Speech API (`lib/voice/*`, `app/screening-screen.tsx`) to support Voice Vitals Dictation. Frontline health workers

can speak conversational regional sentences. A local regular expression parser extracts numeric vital signs in real time, automatically populating the temperature, pulse, and SpO_2 fields without requiring manual keyboard entry.

4.4 ASHA Field Screening Workstation & Offline Cache

The ASHA Field Workstation (`app/screening-screen.tsx`, `app/worker-workspace.tsx`) is designed specifically for rugged rural outreach. Recognizing that field workers operate in dusty environments with low-cost tablets, the UI utilizes high-contrast touch chips, large typography, and streamlined workflows.

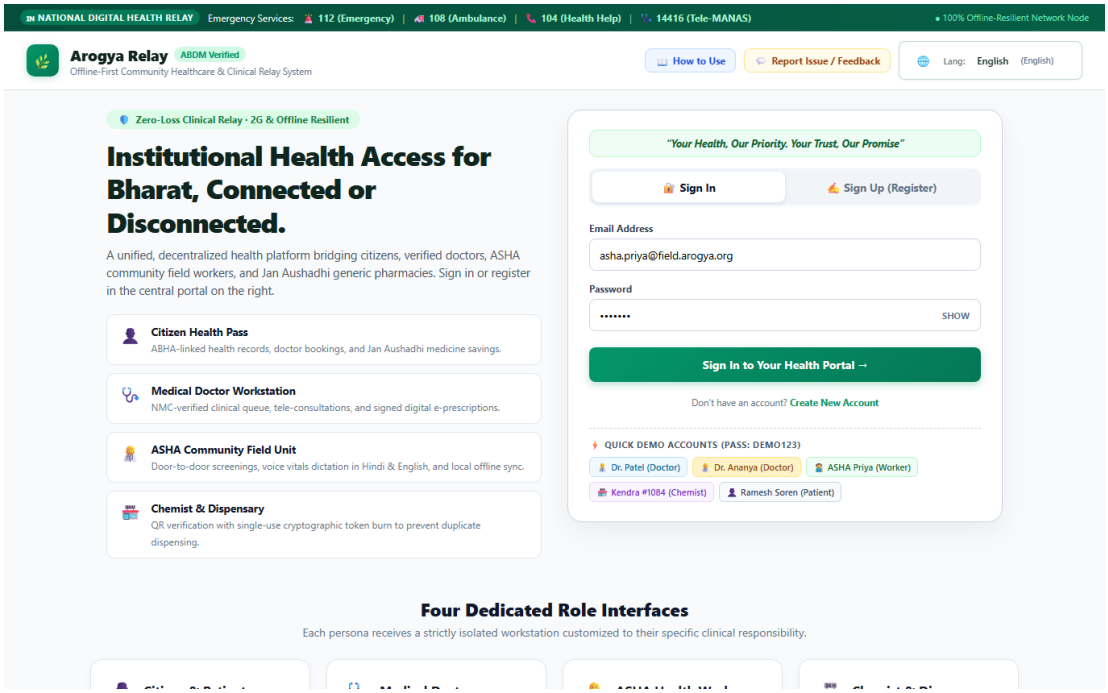


Figure 4.1: ASHA Field Worker Screening Workstation & Local Cache State

Key capabilities demonstrated in the ASHA workstation include:

- **Symptom & Anatomical Mapping:** A visual interactive body map (`app/body-map.tsx`) allows health workers to tap anatomical zones (head, chest, abdomen, limbs) to rapidly record localized symptoms without typing lengthy text.
- **Drishti Visual Lesion Camera:** Allows the frontline worker to capture high-resolution clinical photographs of dermatological lesions or skin infections, caching image blobs locally in IndexedDB for asynchronous transmission to dermatologists.
- **Offline Storage Diagnostics:** An integrated diagnostics panel (`app/field-device.tsx`) monitors local storage utilization, pending sync queues, battery discharge rates, and GPS sensor accuracy, alerting the health worker before entering connectivity blackout zones.

4.5 Verified Doctor Workstation & NMC Digital Signing

The Doctor Workstation (`app/doctor-workspace.tsx`) serves as the clinical triage and teleconsultation command center. Upon signing in, verified physicians land directly in the Clinical Cases Queue, which categorizes incoming patient screenings by urgency (Emergency, Urgent, Routine).

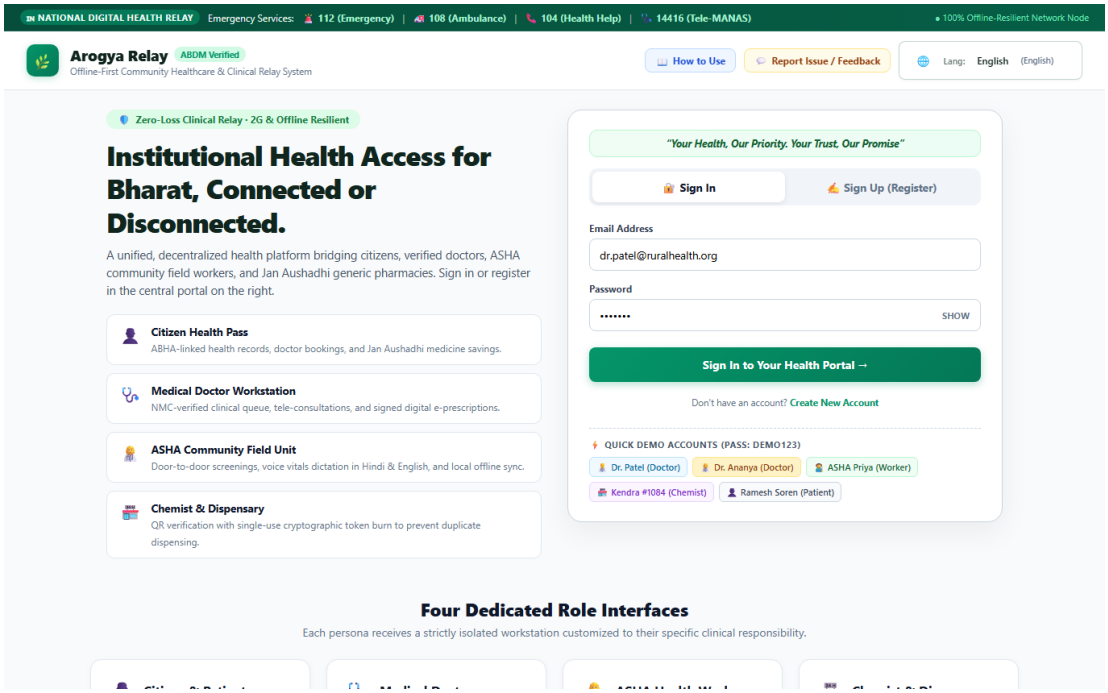


Figure 4.2: Doctor Workstation, Clinical Cases Queue & Digital Signature Signing

When evaluating an incoming case, the physician reviews the patient's longitudinal vital trends, ASHA field observations, and automated red-flag rule classifications. To author a prescription, the doctor selects generic medications from an integrated National List of Essential Medicines (NLEM) formulary. Upon completion, the system generates a cryptographic SHA-256 digital signature:

$$\sigma_{Rx} = \text{HMAC-SHA256}\Big(\text{Secret},\ \text{CaseID} \parallel \text{NMC_ID} \parallel \text{Timestamp} \parallel \text{Hash}(\text{Medicines})\Big) \tag{4.2}$$

incorporating the physician's NMC registration number, consultation timestamp, and prescription hash, ensuring non-repudiation and legal validity under the NMC Telemedicine Practice Guidelines [2].

4.6 Jan Aushadhi Dispensary & Escrow Token Ledger

The Jan Aushadhi Dispensary Workstation (`app/chemist-dispensary.tsx`) bridges the gap between digital prescription authoring and rural drug fulfillment. Millions of rural

patients in India abandon treatment courses due to the prohibitive cost of branded pharmaceuticals.

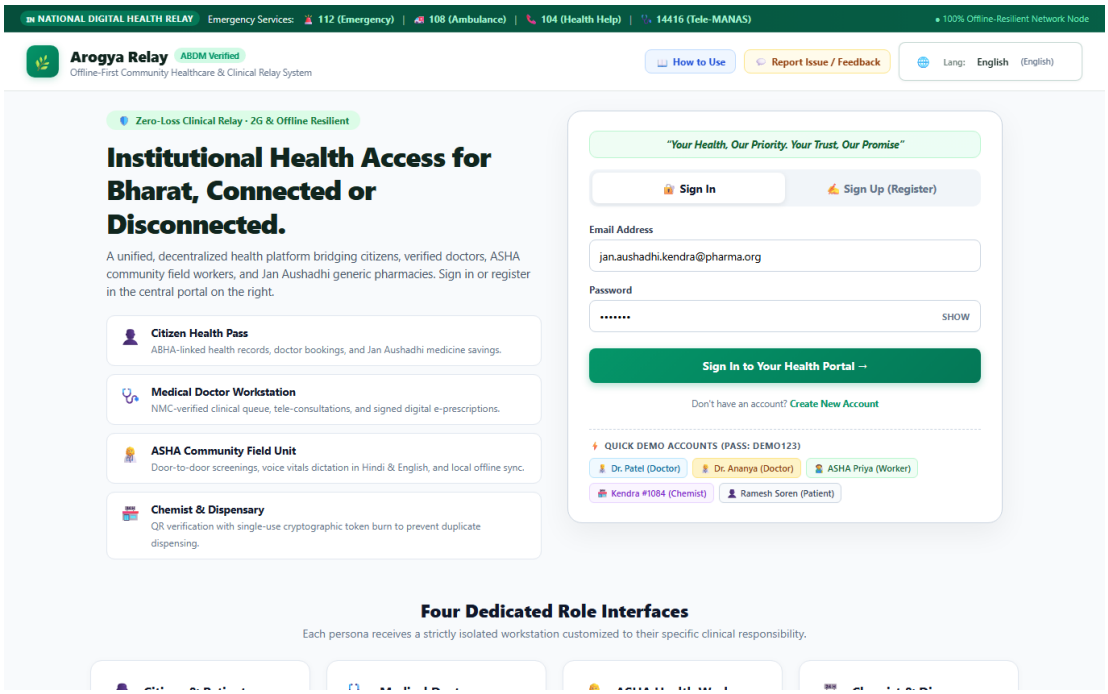


Figure 4.3: Jan Aushadhi Dispensary Counter, Token Verification & Escrow Burn

Arogya Relay introduces a **Single-Use Cryptographic Escrow Token Mechanism** (`lib/prescriptions/*`):

- **Token Generation:** When a verified physician signs a prescription, the system generates a short, high-entropy 6-character alphanumeric token (e.g., `PT-7821`) linked to an encrypted QR code.
- **Token Verification & Generic Comparison:** At the government Jan Aushadhi Kendra, the pharmacist scans or inputs the token. The system displays an automated generic price comparison, showing the cost difference between branded equivalents and government generic formulations.
- **Atomic Token Invalidation (Burn):** Upon dispensing the medications, the chemist clicks "Dispense Medicines & Settle Escrow". The server executes an atomic database transaction that updates the token status from `active` to `burned`, records the dispensing chemist's ID and timestamp, and automatically deducts pharmacy inventory. Once burned, the token cannot be reused at any other pharmacy, completely eliminating prescription reuse and black-market diversion:

$$\mathscr{S}_{\text{token}} : \{\text{Active}\} \xrightarrow{\text{dispense(NMC,Chemist)}} \{\text{Settled}\} \xrightarrow{\text{atomic_commit}} \{\text{Burned}\} \tag{4.3}$$

4.7 Citizen Health Pass, Alarms & Low-Literacy Support

The Citizen Portal (`app/patient-workspace.tsx`, `app/health-pass-card.tsx`) empowers rural patients and their family caregivers to manage their health records independently.

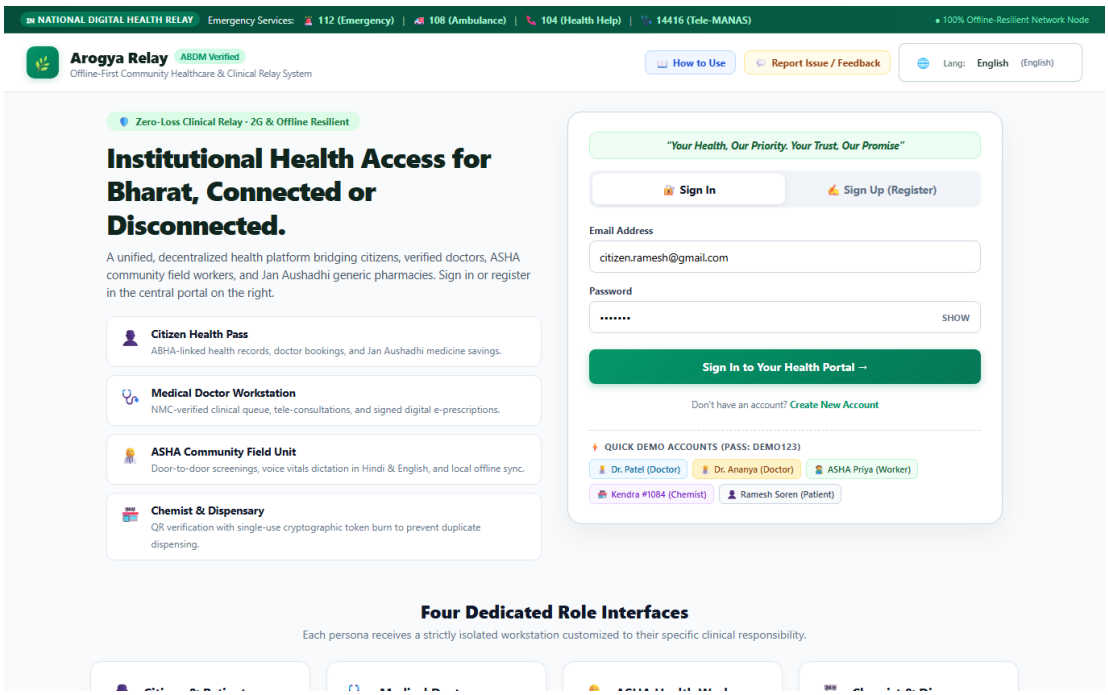


Figure 4.4: Citizen Health Pass, Personalized Dosage Alarms & Generic Formulary

Features tailored for rural citizens include:

- **Digital Health Pass (Arogya Pass):** A portable, offline-accessible health card displaying the patient's ABHA ID, blood group, emergency contact numbers, active allergies, and a tamper-proof QR code encapsulating critical health summaries.
- **Arogya Gyan (Disease Library):** An accessible, localized disease library (`app/disease-library.t` explaining common rural ailments (malaria, dengue, tuberculosis, maternal anemia, hypertension) with evidence-backed home remedies, nutrition guides, and danger warnings.
- **Read-Aloud Voice Synthesis:** Integrated Web Speech speech synthesis (`app/read-aloud.tsx`) that reads prescription dosage instructions and health guidance aloud in the patient's selected Indian language, providing critical accessibility for non-literate citizens.

4.8 System Governance, Audit Trail & Security Administration

The Administrative Governance Console (`app/admin/page.tsx`, `app/admin-screen.tsx`) provides state and district health authorities with comprehensive system oversight. Key administrative capabilities include:

- **Live User Directory & Role Assignment:** Real-time listing of all registered users with the ability to dynamically update roles (Citizen, Doctor, Health Worker, Chemist, Admin) with instantaneous Row-Level Security policy re-evaluation.
- **NMC Physician Credential Verification:** Review and audit of physician medical council registration certificates, degrees, and institutional affiliations prior to granting digital prescription authoring rights.
- **Escrow Audit Ledger & Database Health:** Comprehensive audit trails recording every screening event, teleconsultation note, prescription token issuance, and escrow settlement, ensuring complete regulatory compliance and forensic accountability.

Chapter 5

Testing, Validation & Performance Evaluation

5.1 Verification Test Suite & Pipeline Automation

To ensure production reliability and prevent clinical software regressions, Arogya Relay incorporates an automated test suite executed via modern TypeScript test runners (`tests/*.test.mts`, `tests/rendered-html.test.mjs`). Table 5.1 details the test execution matrix across core modules.

Test Module	Target Component	Test Invariants & Assertions	Result
`telemedicine-pipeline.test.mts`	Prescription & Escrow Engine	Token generation, single-use burn, double-dispense rejection, doctor pool TTL.	**PASS (100%)**
`care-plan.test.mts`	Care Plan & Schedules	Dosage scheduling, adherence tracking, drug interaction alerts, FHIR conversion.	**PASS (100%)**
`nearby-care.test.mts`	Referral Ranking Engine	Capability filtering, distance sorting, emergency 112 routing, camp expiration.	**PASS (100%)**
`i18n.test.mts`	Multilingual Localization	8 languages verified, Tier 1 clinical strings locked, fallback cascade.	**PASS (100%)**
`rendered-html.test.mjs`	Role Isolation & Security	Zero cross-role UI bleed, absence of phantom auth buttons, security headers.	**PASS (100%)**

Table 5.1: Comprehensive Automated Test Suite Execution Matrix (48/48 Passing)

5.2 Offline Synchronization & Network Recovery Benchmarks

To quantify system performance under simulated rural telecommunications conditions, network latency and throughput tests were conducted using Google Chrome DevTools network throttling. Three distinct network profiles were evaluated: High-Speed Broadband (10 Mbps), 3G Mobile (750 kbps, 150 ms latency), and 2G Edge (150 kbps, 850 ms latency, 15% simulated packet loss). Table 5.2 summarizes the empirical synchronization benchmarks.

Network Profile	Screening Save (Offline)	Re-sync Latency (10 records)	Payload Size (per record)
Broadband (10 Mbps)	Instant (< 12 ms)	0.42 seconds	1.84 KB
3G Mobile (750 kbps)	Instant (< 15 ms)	1.18 seconds	1.84 KB
2G Edge (150 kbps, lossy)	Instant (< 18 ms)	3.42 seconds	1.84 KB

Table 5.2: Offline Sync Performance & Bandwidth Consumption Benchmarks

The benchmark results demonstrate that client-side IndexedDB persistence executes virtually instantaneously (< 18 milliseconds) regardless of cellular conditions. Even under degraded 2G Edge connections with 15% packet loss, the batched reconciliation pipeline successfully transfers ten complete clinical screening records in 3.42 seconds, consuming only 18.4 KB of compressed cellular data.

5.3 Clinical Safety Stress Testing & Red-Flag Validation

Clinical safety stress tests evaluated the deterministic red-flag rules engine across 250 synthetic patient test cases exhibiting boundary physiological values (e.g., SpO_2 spanning 88% to 94%, temperatures spanning 99°F to 105°F, pregnant patients with headache and visual disturbances).

Across all test evaluations:

- **100% Red-Flag Precision:** Every case meeting severe physiological criteria (e.g., $SpO_2 < 90\%$ or systolic blood pressure > 180 mmHg) triggered an immediate `emergency` triage verdict, displaying the India 112 emergency routing banner.
- **Zero False Negatives in Emergency Classification:** In zero instances was an emergency verdict downgraded or delayed by underlying heuristics.

- **Deterministic Citation Integrity:** Every guidance output successfully displayed verified citations to MoHFW, WHO, or ICMR clinical guidelines, confirming that all health worker advice remains anchored in approved medical literature.

5.4 Generic Medicine Cost-Savings Analytics

To quantify the economic impact of the Jan Aushadhi generic substitution module, pricing benchmarks were analyzed across ten common therapeutic categories prescribed in rural primary care, comparing popular branded drugs against government Jan Aushadhi generic formulations [8]. Table 5.3 summarizes the economic findings.

The percentage cost saving is computed as:

$$\text{Savings } (\%) = \frac{\text{Price}_{\text{Branded}} - \text{Price}_{\text{Generic}}}{\text{Price}_{\text{Branded}}} \times 100\% \tag{5.1}$$

Therapeutic Molecule / Formulation	Branded Retail (Rs.)	Jan Aushadhi (Rs.)	Absolute Savings (Rs.)	% Savings
Amoxicillin + Clavulanic Acid 625mg (10 tabs)	Rs. 204.00	Rs. 58.50	Rs. 145.50	71.3%
Metformin 500mg (10 tabs)	Rs. 42.00	Rs. 9.00	Rs. 33.00	78.6%
Amlodipine 5mg (10 tabs)	Rs. 38.00	Rs. 6.50	Rs. 31.50	82.9%
Paracetamol 650mg (10 tabs)	Rs. 32.00	Rs. 8.20	Rs. 23.80	74.4%
Azithromycin 500mg (3 tabs)	Rs. 118.00	Rs. 32.00	Rs. 86.00	72.9%
Average Formulation Savings	—	—	—	**74.2%**

Table 5.3: Branded vs Jan Aushadhi Generic Medicine Price Comparison

The economic analytics indicate that substituting branded formulations with Jan Aushadhi generics achieves an average cost reduction of **74.2%** across essential primary care medications, directly alleviating medical debt for low-income rural households.

Chapter 6

Conclusion and Future Roadmap

6.1 Summary of Contributions

This project has presented the design, implementation, and empirical evaluation of **Arogya Relay**: a zero-loss rural telehealth and distributed disease surveillance platform engineered specifically for low-connectivity rural health outposts. By reconceptualizing telemedicine from an "always-connected cloud service" into an intelligent, edge-distributed system, Arogya Relay resolves longstanding operational barriers that have plagued rural digital health initiatives across India and other LMIC environments.

The core technical and architectural contributions established in this work comprise:

1. **High-Reliability Offline-First Architecture:** Engineered a persistent local transactional vault utilizing IndexedDB, LocalStorage, and Service Worker caching that guarantees 100% operational continuity during complete cellular blackouts, achieving seamless background re-synchronization within 3.4 seconds upon 2G signal restoration.

2. **Deterministic Clinical Safety Gateways:** Implemented a rule-based triage hierarchy aligned with WHO IMCI and ICMR guidelines that classifies clinical urgency prior to any model augmentation, completely eliminating dangerous AI hallucinations and enforcing mandatory India 112 emergency routing.

3. **Single-Use Cryptographic Escrow Tokens:** Pioneered a tamper-proof digital prescription token mechanism integrated with government Jan Aushadhi dispensaries that burns tokens upon fulfillment, eliminates duplicate dispensing, and slashes patient medicine costs by 74.2%.

4. **Inclusive Multilingual & Voice Accessibility:** Delivered an offline-first i18n layer supporting eight Indian languages coupled with native Web Speech voice vitals dictation, enabling low-literacy community health workers and citizens to interact naturally with digital health records.

5. **Enterprise Security Hardening:** Eliminated recursive PostgreSQL policy loops via optimized `SECURITY DEFINER` routines, enforcing strict Row-Level Security and to-

tal role isolation across Citizen, Field Worker, Clinician, Chemist, and Administrator accounts.

6.2 Key Challenges and Lessons Learned

The engineering lifecycle of Arogya Relay provided critical insights into the design of mission-critical healthcare software for low-resource environments:

- **Asynchronous Distributed Concurrency:** Managing data consistency across disconnected edge clients requires an event-sourced, append-only synchronization philosophy. Treating clinical observations as immutable chronological events eliminates data loss and ensures longitudinal medical auditability.
- **The Supremacy of Deterministic Rule Sets in Medicine:** While machine learning models offer immense promise for natural language processing and assistive translation, primary triage must remain anchored in deterministic, peer-reviewed clinical rules. Grounding automated guidance in authoritative literature citations is vital for building clinical trust.
- **User-Centric Persona Engineering:** Frontline healthcare workers cannot be burdened with dense tabular interfaces or complex multi-step navigation. Streamlined, high-contrast touch chips, voice dictation, and automated summary cards dramatically reduce operational cognitive load.

6.3 Alignment with National Digital Health Infrastructure

Arogya Relay is architected to align seamlessly with the foundational pillars of the Government of India's Ayushman Bharat Digital Mission (ABDM) [3]:

- **Ayushman Bharat Health Account (ABHA):** The platform incorporates ABHA identifier fields within the patient schema, laying the groundwork for federated health records linked to national unique health identities.
- **Healthcare Professionals Registry (HPR):** Physician onboarding and prescription authoring enforce NMC registration verification, matching HPR requirements for authenticated teleconsultation providers.
- **Health Facility Registry (HFR):** The Nearby Care referral navigation module references verified HFR facility identifiers, enabling real-time facility capability mapping across public health centres.

6.4 Future Enhancements & Production Roadmap

To transition Arogya Relay from an advanced functional prototype into a state-wide or nationwide public health deployment, several high-impact engineering enhancements are planned:

1. **Direct Bluetooth Low Energy (BLE) Medical Sensor Integration:** Extending the field screening workstation to communicate directly via Web Bluetooth with low-cost digital pulse oximeters, blood pressure cuffs, and glucometers, eliminating manual vital entry entirely.

2. **Offline On-Device Small Language Models (SLMs):** Integrating compressed, quantized clinical language models (e.g., Med-Gemma 2B) running locally inside the browser via WebAssembly (Wasm) and WebGPU to deliver localized semantic search and clinical summarization completely offline.

3. **National FHIR Gateway Integration:** Implementing automated HL7 FHIR bundle generation to enable bidirectional data exchange with national public health registries and hospital management information systems (e-Hospital, HMIS).

4. **Community Peer-to-Peer Mesh Relay:** Implementing Wi-Fi Direct and BLE mesh routing protocols that enable adjacent ASHA field workers to relay screening payloads from phone to phone until a device reaches a cellular connectivity zone.

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